

Important Information, Please read carefully.

RINALIX[®]
INDAPAMIDE

Pharmacode Area

Tablets

Composition

Each tablet contains: Indapamide 2.5 mg

Pharmacodynamics

Indapamide is a diuretic with actions similar to those of the thiazide diuretics. It acts directly on the kidney to increase the excretion of sodium chloride and water; it also increases the excretion of K^+ .

Pharmacokinetics

Indapamide is rapidly and completely absorbed from the gastro-intestinal tract. Peak blood levels are obtained after 1 to 2 hours. Indapamide is strongly and reversibly bound to plasma protein and to erythrocytes. A minimum of 70% of a single oral dose is eliminated by the kidneys and an additional 23% by the gastrointestinal tract. Indapamide is metabolised with 7% of the unchanged product found in the urine during the 48 hours following administration. The elimination half-life of Indapamide is approximately 15-18 hours. Indapamide is strongly and reversibly bound to plasma protein and to erythrocytes.

Indications

Rinalix is indicated for the treatment of essential hypertension, alone or in combination with other antihypertensive drugs.

Dosage and Administration

To be administered orally.

Hypertension: 2.5 mg daily, to be taken in the morning. The action of Indapamide is progressive and the reduction in blood pressure may continue and not reach a maximum until several months after the initiation of therapy. A larger dose than 2.5 mg Indapamide is not recommended as there is no appreciable antihypertensive effect but a diuretic effect may become apparent.

Contraindications

Anuria, stroke, recent cerebrovascular accident, severe hepatic insufficiency, known hypersensitivity to indapamide or to other sulfonamide-derived drugs.

Precautions / Warnings

Hypokalemia, Hyponatremia, and Other Fluid and Electrolyte imbalances: Periodic determinations of serum electrolytes should be performed at appropriate intervals. Patients should be observed for clinical signs of fluid or electrolyte imbalance, such as hyponatremia, hypochloremic alkalosis, or hypokalemia.

Hyperuricemia and Gout: Serum concentrations of uric acid was reported to increase in patients treated with indapamide, and frank gout may be precipitated in certain patients receiving indapamide. Serum concentrations of uric acid should therefore be monitored periodically during treatment.

Renal Impairment: Indapamide should be used with caution in patients with severe renal disease, as reduced plasma volume may exacerbate or precipitate azotemia. If progressive renal impairment is observed, withholding or discontinuing diuretic therapy should be considered. Renal function tests should be performed periodically during treatment with indapamide.

Impaired Hepatic Function: Indapamide should be used with caution in patients with impaired hepatic function or progressive liver disease, since minor alterations of fluid and electrolyte balance may precipitate hepatic coma.

Calcium Excretion: Calcium excretion is decreased by diuretics pharmacologically related to indapamide. Prolonged treatment with drugs pharmacologically related to indapamide may in rare instances be associated with hypercalcemia and hypophosphatemia secondary to physiologic changes in the parathyroid gland; however, the common complications of hyperparathyroidism, such as renal lithiasis, bone resorption, and peptic ulcer, have not been seen. Treatment should be discontinued before tests for parathyroid function are performed. Like the thiazides, indapamide may decrease serum PBI (protein-bound iodine) levels without signs of thyroid disturbance.

Choroidal Effusion, Acute Myopia, and Secondary Angle-Closure Glaucoma: Sulfonamide or sulfonamide derivative drugs can cause an idiosyncratic reaction resulting in choroidal effusion with visual field defect, transient myopia and acute angle-closure glaucoma. Symptoms may include acute onset of decreased visual acuity or ocular pain and typically occur within hours to weeks of drug initiation. Untreated acute angle-closure glaucoma can lead to permanent vision loss. The primary treatment is to discontinue drug intake as rapidly as possible. Prompt medical or surgical treatments may need to be considered if the intraocular pressure remains uncontrolled. Risk factors for developing acute angle-closure glaucoma may include a history of sulfonamide or penicillin allergy.

Use in Elderly: Plasma potassium and urate concentrations should be monitored when indapamide is used in elderly.

Usage in Pregnancy: Reproduction studies have been performed in animals and have revealed no evidence of impaired fertility or harm to the fetus due to indapamide. There are, however, no adequate and well-controlled studies in pregnant women. Moreover, diuretics are known to cross the placental barrier and appear in cord blood. Therefore, the drug should be used during pregnancy only if clearly needed. There may be hazards associated with this use such as fetal or

neonatal jaundice, thrombocytopenia, and possibly other adverse reactions that have occurred in the adult.

Nursing Mothers: It is not known whether this drug is excreted in human milk. Because most drugs are excreted in human milk, if use of this drug is seemed essential, the patient should stop nursing.

Undesirable effects

Summary of safety profile

The most commonly reported adverse reactions are hypokalaemia, hypersensitivity reactions, mainly dermatological, in subjects with a predisposition to allergic and asthmatic reactions and maculopapular rashes.

Tabulated summary of adverse reactions

The following undesirable effects have been observed with indapamide during treatment ranked under the following frequency:

Very common ($\geq 1/10$); common ($\geq 1/100$ to $< 1/10$); uncommon ($\geq 1/1,000$ to $< 1/100$); rare ($\geq 1/10,000$ to $< 1/1,000$); very rare ($\geq 1/100,000$ to $< 1/10,000$), not known (cannot be estimated from the available data).

MedDRA System Organ Class	Undesirable Effects	Frequency
Blood and the lymphatic System Disorders	Agranulocytosis	Very rare
	Aplastic anaemia	Very rare
	Haemolytic anaemia	Very rare
	Leucopenia	Very rare
	Thrombocytopenia	Very rare
Metabolism and Nutrition Disorders	Hypercalcaemia	Very rare
	Hypokalaemia	Common
	Hyponatraemia	Uncommon
	Hypochloraemia	Rare
	Hypomagnesaemia	Rare
Nervous System disorders	Vertigo	Rare
	Fatigue	Rare
	Headache	Rare
	Paraesthesia	Rare
	Syncope	Not known
Eye disorders	Myopia	Not known
	Blurred vision	Not known
	Visual impairment	Not known
	Acute angle-closure glaucoma	Not known
	Choroidal effusion	Not known
Cardiac Disorders	Arrhythmia	Very rare
	Torsade de pointes (potentially fatal)	Not known

Vascular Disorders	Hypotension	Very rare
Gastrointestinal Disorders	Vomiting	Uncommon
	Nausea	Rare
	Constipation	Rare
	Dry mouth	Rare
	Pancreatitis	Very rare
Hepatobiliary Disorders	Abnormal hepatic function	Very rare
	Possibility of onset of hepatic encephalopathy in case of hepatic insufficiency	Not known
	Hepatitis	Not known
Skin and Subcutaneous Tissue Disorder	Hypersensitivity reactions	Common
	Maculopapular rashes	Common
	Purpura	Uncommon
	Angioedema	Very rare
	Urticaria	Very rare
	Toxic epidermal necrolysis	Very rare
	Stevens-Johnson Syndrome	Very rare
	Possible worsening of pre-existing acute disseminated lupus erythematosus	Not known
	Photosensitivity reactions	Not known
Renal and Urinary Disorders	Renal failure	Very rare
Musculoskeletal and Connective Tissue Disorders	Muscle spasms	Not known
	Muscular weakness	Not known
	Myalgia	Not known
	Rhabdomyolysis	Not known
Reproductive system and breast disorders	Erectile dysfunction	Uncommon
Investigations	Electrocardiogram QT prolonged	Not known
	Blood glucose increased	Not known
	Blood uric acid increased	Not known
	Elevated liver enzyme levels	Not known

Drug Interactions

*Lithium: In general, diuretics should not be given concomitantly with lithium because they reduce its renal clearance and add a high risk of lithium toxicity.

*Fenoxedil, lidoflazine, prenylamine and vincamine, non-antiarrhythmic drugs inducing wave burst arrhythmia (hypokalemia or even bradycardia and a preexisting long Q-T interval are predisposing factors).

*Tienilic Acid: Acute renal failure, (intratubular precipitation of urates related to the

uricosuric effect of tienilic acid and to the state of dehydration due to other diuretics).

Other Antihypertensives: Indapamide may add to or potentiate the action of other antihypertensive drugs.

Norepinephrine: Indapamide, like the thiazides, may decrease arterial responsiveness to norepinephrine, but this diminution is not sufficient to preclude effectiveness of the pressor agent for therapeutic use.

NSAIDs (systemic route): Acute renal insufficiency in the dehydrated patient (reduction in glomerular filtration due to the inhibition of vasodilator prostaglandins by the NSAIDs). Hydrate the patient and monitor renal function at the start of treatment.

Metformin: Lactic acidosis due to metformin triggered by possible functional renal failure related to the diuretics. Do not use metformin when blood creatinine levels exceed 15 mg/litre (135 micromoles/litre) in men and 12 mg/litre (110 micromoles/litre) in women.

Iodinated contrast media: In cases of dehydration caused by diuretics, there is an increased risk of acute renal failure, in particular when high doses of iodinated contrast media are used. Rehydration before administration of the iodinated product.

Antiarrhythmics inducing wave burst arrhythmia: sotalol, amiodarone, bepridil and quinidine-type anti-arrhythmic drugs (class Ia), Wave burst arrhythmia (hypokalemia and even bradycardia and pre-existing long Q-T interval are predisposing factors).

Prevention and, if necessary, correction of the hypokalemia, monitoring of the Q-T interval; do not give antiarrhythmic drugs in cases of wave burst arrhythmia (cardiac pacing).

Other hypokalemia drugs: amphotericin B (IV route), glucocorticoids and mineralocorticoids (systemic route), tetracosactide, stimulant laxatives: increased risk of hypokalemia (additive effect).

Hyperkalemia diuretics (amiloride, canrenone, spironolactone, triamterene). Rational drug combinations which are useful in certain patients do not exclude the occurrence of hypokalemia or hyperkalemia in particular in patients with renal failure and diabetes.

Digitalis glycosides: Hypokalemia potentiates the toxic effects of digitalis glycosides.

*Inadvisable combinations

Symptoms and Treatment of Overdosage

Symptoms of overdosage include nausea, vomiting, weakness, gastrointestinal disorders and disturbances of electrolyte balance. In severe instances, hypotension and depressed respiration may be observed. If this occurs, support of respiration and cardiac circulation should be instituted. There is no specific antidote. An evacuation of the stomach is recommended by emesis and gastric lavage after which the electrolyte and fluid balance should be evaluated carefully.

Shelf-life

The expiry date is indicated on the packaging.

Description

Pink color, round and deep convex film-coated tablet, 5.5 mm in diameter in packs of 60's. 15's tablets per blister.

Storage

Store at room temperature below 30°C. Protect from light and moisture.

KEEP OUT OF REACH OF CHILDREN

JAUHI DARI KANAK-KANAK.

For further information, please consult your pharmacist or physician

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Manufactured and Marketed by

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